



Marloo Gardens Aged Care

Restrictive Practices and Behaviour Support Policy

Authorisation, consent, minimisation and elimination of restrictive practices

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Document control

This document is controlled by the Clinical Nurse Consultant, Behaviour Support. Printed copies are uncontrolled and must be checked against the document register before use. Proposed amendments are submitted to the document owner and considered by the Clinical Governance Committee at its next scheduled meeting.

Version history

Version	Date	Author	Summary of change
3.0	8 Aug 2024	L. Ferreira	Aligned authorisation with the restrictive practices substitute decision-maker requirements.
4.0	19 May 2025	L. Ferreira	Added the elimination plan requirement at Part 5.
4.2	3 Feb 2026	T. Nguyen	Added environmental restraint guidance for secure units.
4.3	21 Aug 2026	T. Nguyen	Strengthened the least-restrictive-alternative evidence requirement at 3.2.

Distribution and accessibility

The current version is published on the provider intranet and is available in hard copy at each nurses' station. Alternative formats, including large print and interpreted summaries, are provided on request to the document owner.

Audience	Method	Frequency
All care and clinical workers	Intranet and induction pack	On publication
Registered and enrolled nurses	Clinical handover briefing	On publication
Consumer Advisory Body	Emailed summary	Quarterly
Contracted and agency staff	Agency portal	Before first shift

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Part 1 Purpose, position and definitions

A restrictive practice limits a person's rights or freedom of movement. Marloo Gardens Aged Care starts from the position that restrictive practices cause harm, that their use represents a failure to meet a need by other means, and that the provider's objective is elimination rather than management.

A restrictive practice may be used only as a last resort, to prevent harm, after alternatives have been tried and documented, for the shortest possible time, in the least restrictive form, with informed consent, and under a current behaviour support plan. Every one of these conditions must be met; meeting most of them is not compliance.

1.1 The five forms

Form	Meaning	Examples
Chemical restraint	A medicine used to influence behaviour, other than to treat a diagnosed condition	Antipsychotic given for agitation; sedative given at night to reduce walking
Physical restraint	Physical or bodily force preventing or restricting movement	Bed rails; lap belts; a worker holding a person
Mechanical restraint	A device preventing or restricting movement	Restraining chair; wheelchair brake applied to prevent standing
Environmental restraint	Restricting free access to the person's environment	Locked doors; keypad exits; removing a walking frame
Seclusion	Solitary confinement in a room the person cannot freely exit	Locking a person alone in a room at any time

1.2 What is not a restrictive practice

A practice used to deliver necessary clinical care with consent, such as a splint after surgery, is not a restrictive practice provided it is not used to influence behaviour. A locked medication room is not an environmental restraint. Where the purpose is uncertain, the practice is treated as restrictive until the Clinical Nurse Consultant determines otherwise.

Part 2 Understanding behaviour before responding to it

Changed behaviour is communication. Before any restrictive practice is considered, the provider must seek to understand what the person is communicating and what unmet need lies behind it.

2.1 Assessment for reversible causes

- Pain, assessed with a validated tool appropriate to the person's communication ability.
- Delirium, infection, constipation, urinary retention and dehydration.
- Medicine side effects, interactions and recent changes.
- Sensory impairment, including missing glasses, hearing aids or dentures.
- Hunger, thirst, fatigue, temperature and need for the toilet.
- Fear, boredom, grief, noise, overstimulation and unfamiliar workers.

2.2 Pain as the most commonly missed cause

Pain is the single most commonly missed cause of changed behaviour in people living with dementia. Where a person cannot report pain reliably, an observational assessment tool must be used and a trial of analgesia considered before any psychotropic medicine is contemplated. The outcome of the trial is documented.

2.3 Behaviour support planning

Every person who experiences changed behaviour has a behaviour support plan developed with them, their supporters and the multidisciplinary team. The plan records what the behaviour is, when and where it occurs, what appears to trigger it, what has helped, and what to do. It is reviewed at least every 12 months, and within 5 days of any new restrictive practice.

Part 3 Authorisation and consent

A restrictive practice must not be used before it has been authorised and consented to, except in a genuine emergency under Part 4.

3.1 Who must consent

Informed consent must be obtained from the person, or where the person lacks capacity for the decision, from the restrictive practices substitute decision-maker recognised under the law of the relevant State or Territory. Consent must be specific to the practice, not general, and must be recorded in writing with the date, the person consenting and their authority to do so.

3.2 Evidence of least restrictive alternatives

The authorisation record must set out what alternatives were tried, over what period, and why each was insufficient. A statement that alternatives 'were tried' without detail is not evidence and will be treated as a non-conformity at audit. The alternatives considered must be specific to this person, not a generic list.

3.3 Contents of an authorisation

Element	Requirement
The practice	The specific form, device or medicine, and the exact circumstances of use
The purpose	The harm being prevented, stated specifically
Alternatives	What was tried, for how long, and why each was insufficient
Duration	A defined end date, never open-ended
Monitoring	How often the person is observed and what is recorded
Review date	No more than 3 months from authorisation, and sooner if circumstances change
Consent	Who consented, their authority, and the date

3.4 Prohibited practices

Seclusion is prohibited at Marloo Gardens in all circumstances. Any practice used as punishment, for the convenience of workers, or in place of adequate staffing is prohibited and is a reportable incident.

Part 4 Emergency use and reporting

A restrictive practice may be used without prior authorisation only where there is an imminent risk of serious harm to the person or another, and no less restrictive option is available at that moment.

4.1 Obligations following emergency use

Obligation	Timeframe	Responsible
Notify the registered nurse in charge	Immediately	Worker involved
Notify the substitute decision-maker	Within 24 hours	Facility Manager
Record as a reportable incident	Within 24 hours	Facility Manager
Clinical review of the person	Within 24 hours	Registered nurse
Behaviour support plan updated or created	Within 5 days	Clinical Nurse Consultant
Authorisation obtained or the practice ceased	Within 5 days	Clinical Nurse Consultant
Reported to the Clinical Governance Committee	At the next meeting	Clinical Nurse Consultant

4.2 Unauthorised use is a reportable incident

Use of a restrictive practice without consent, outside an authorisation, beyond its end date, or in a prohibited form is a Priority 1 reportable incident and must be notified to the Commission within 24 hours. This applies whether the use was deliberate, mistaken or the result of a lapsed authorisation.

4.3 Monitoring during use

A person subject to physical or mechanical restraint must be observed at intervals no longer than 15 minutes, with skin integrity, circulation, positioning, comfort, hydration and toileting needs checked and recorded. The restraint must be released at least every two hours for repositioning and movement unless a shorter interval is clinically indicated.

Part 5 Minimisation, review and elimination

Every authorised restrictive practice must have an elimination plan from the day it begins. Continuation is never the default; the question at every review is what would have to change for this practice to stop.

5.1 Review cycle

Each authorisation is reviewed at least every three months by the multidisciplinary team, and the review must record whether the practice is still necessary, whether a less restrictive alternative has become available, and what progress has been made toward elimination. A review that simply renews the practice without this reasoning is not a review.

5.2 Elimination plan

The elimination plan states the target date for cessation, the steps toward it, and who is accountable for each. Where a practice has been in place for more than 12 months, the plan must be escalated to the Clinical Governance Committee with an explanation. All practices in place for more than 12 months must be escalated and re-examined by 31 January 2027.

5.3 Reporting and transparency

The prevalence of each form of restrictive practice is reported to the Board quarterly and to the National Aged Care Mandatory Quality Indicator Program as required. Prevalence is published to residents and supporters in the quarterly quality report. The annual restrictive practices reduction report is due to the Board by 31 August 2027.

Part 5A Secure units and environmental restraint

A locked or keypad-controlled unit is an environmental restraint for every person living in it, and each person requires an individual authorisation and consent. Living in a secure unit is not itself consent to environmental restraint.

5A.1 Entry to a secure unit

Entry requires a documented assessment that the person is at risk of harm from leaving unaccompanied, consideration of alternatives including supervised walking programs and wearable location technology, consent from the person or their substitute decision-maker, and a review date no more than three months away.

5A.2 Life within the unit

A person living in a secure unit retains every other right, including access to outdoor space each day, visitors at times of their choosing, and participation in activities outside the unit with support. Restricting these is a further restrictive practice requiring separate authorisation.

Part 6 Governance and accountability for restrictive practices

The Board of Marloo Gardens Aged Care holds ultimate accountability for restrictive practices and discharges that accountability through the Clinical Governance Committee. The Board receives a standing report at each ordinary meeting covering performance against the indicators set out in Part 9, together with any matter the Clinical Nurse Consultant, Behaviour Support considers requires the Board's attention before the next reporting cycle.

The Clinical Governance Committee meets monthly. Its quorum is four members and must include at least one registered nurse and one member independent of the operational management line. Minutes are retained for seven years and are made available to the Aged Care Quality and Safety Commission on request.

6.1 Allocation of responsibilities

Responsibility for the requirements of this document is allocated as set out below. Allocation of a responsibility does not relieve any worker of their own professional and legal obligations.

Role	Responsibility
Board	Reviews restrictive practice prevalence and elimination progress quarterly.
Clinical Nurse Consultant, Behaviour Support	Owns this document; authorises practices; maintains the register; leads elimination planning.
Prescribers	Assess for reversible causes before prescribing psychotropics; review continuing need every 3 months.
Facility Manager	Notifies substitute decision-makers; records emergency use as a reportable incident.
Registered nurses	Complete monitoring observations; escalate lapsed authorisations; assess for pain and delirium.
Care workers	Apply behaviour support strategies; report any use of restraint immediately.

6.2 Delegation

The Clinical Nurse Consultant, Behaviour Support may delegate an operational function under this document in writing, recording the scope and duration of the delegation in the delegations register. A delegation must not exceed twelve months without review, and the delegate must hold the qualifications and current registration the function requires. The Clinical Nurse Consultant, Behaviour Support remains accountable for a delegated function.

6.3 Conflicts of interest

A worker who has a personal, financial or familial interest that could reasonably be perceived to influence a decision made under this document must declare that interest to the Clinical Nurse Consultant, Behaviour Support before taking part in the decision. Declared interests are recorded in the conflicts register and reviewed by the Clinical Governance Committee each quarter.

Part 7 Workforce competence and training

No worker may perform a function governed by this document until they have completed the training set out in this Part and been assessed as competent. The Clinical Nurse Consultant, Behaviour Support maintains a training matrix mapping every role to its mandatory modules, and the workforce management system prevents rostering a worker to a function for which their competency has lapsed.

Module	Audience	Frequency	Assessment
Restrictive practices law and authorisation	All clinical workers	Annually	Scenario assessment
Behaviour support and de-escalation	All care and clinical workers	Annually	Direct observation
Pain assessment in people with dementia	Registered and enrolled nurses	Annually	Tool-based assessment
Recognising delirium	All clinical workers	Annually	Online assessment
Safe use of bed rails and lap belts	Care workers	Annually	Direct observation

7.1 Induction

New workers complete the restrictive practices component of induction before their first unsupervised shift. Induction covers this document, the escalation pathway in Part 5, and the worker's obligation to report. A worker who has not completed induction may work only under the direct supervision of a competent worker.

7.2 Competency assessment and remediation

Competency is assessed by direct observation against the criteria at Appendix B, not by attendance alone. A worker assessed as not yet competent is placed on a supervised practice plan for a defined period, reassessed at the end of that period, and stood down from the relevant function if the second assessment is not satisfactory.

The Clinical Nurse Consultant, Behaviour Support reports training completion rates to the Clinical Governance Committee monthly. Where completion for any mandatory module falls below 95 per cent, the Clinical Nurse Consultant, Behaviour Support must provide the Clinical Governance Committee with a written recovery plan at the next meeting.

Part 8 Records, privacy and information management

Records created under this document form part of the care record and are managed in accordance with the provider's Records Management Policy and the Privacy Act 1988 (Cth). Entries must be contemporaneous, factual, legible and attributed to an identified author, and must not be altered after the fact except by a dated and attributed addendum.

8.1 Retention

Record	Minimum retention	Custodian
Restrictive practice authorisations and consents	7 years after cessation	Clinical Nurse Consultant
Behaviour support plans	Life of the care record	Health Information Manager
Monitoring observation records	7 years	Health Information Manager
Restrictive practices register	Permanent	Clinical Nurse Consultant

8.2 Access and disclosure

An individual, or their supporter where authorised, may access records held about them on written request. Requests are actioned within 30 days. Personal information is disclosed outside the provider only with consent, or where disclosure is required or authorised by law, and every such disclosure is recorded in the disclosure register.

8.3 Information security

Access to the clinical information system is role-based and individually credentialled. Shared logins are prohibited. Access is revoked on the day a worker's engagement ends, and access logs are reviewed quarterly for anomalous patterns. A suspected data breach is escalated immediately under the Notifiable Data Breaches scheme.

Part 9 Monitoring, measurement and continuous improvement

Performance under this document is measured against the indicators below. The Clinical Nurse Consultant, Behaviour Support reports results to the Clinical Governance Committee monthly and to the Board quarterly. An indicator that falls outside its target for two consecutive months triggers a documented improvement action with a named owner and a due date.

Indicator	Target	Source	Frequency
Authorisations with documented least-restrictive evidence	100%	Register audit	Monthly
Practices reviewed within 3 months	100%	Register audit	Monthly
Emergency use with authorisation or cessation within 5 days	100%	Incident records	Monthly
Residents subject to physical restraint	below 5%	Quality indicator extract	Quarterly
Residents prescribed an antipsychotic	below 10%	Quality indicator extract	Quarterly
Practices in place beyond 12 months	Reducing each quarter	Register	Quarterly

9.1 Internal audit

The Clinical Nurse Consultant, Behaviour Support conducts a documented internal audit of compliance with this document at least twice each year, using the tool at Appendix C. Findings are rated as conformity, opportunity for improvement, minor non-conformity or major non-conformity. A major non-conformity must be reported to the Clinical Governance Committee within five business days of identification.

9.2 Continuous improvement register

Improvement actions arising from audits, incidents, complaints, feedback and consumer advisory input are recorded in the continuous improvement register with an owner, a due date and an evidence field. The register is reviewed at each committee meeting and overdue items are escalated to the Board.

9.3 Consumer and workforce input

The provider seeks the views of individuals receiving care, their supporters and the workforce on the operation of this document at least annually, through the consumer experience survey, resident and relative meetings and workforce feedback channels. Results and the provider's response are published in the quarterly quality report.

Part 10 Risk management

Risks to the delivery of the requirements in this document are assessed using the enterprise risk matrix, which rates a risk on the likelihood of occurrence against the consequence for the individual receiving care. Risks rated high or extreme are recorded on the strategic risk register and reviewed by the Board each quarter.

Risk	Rating	Existing controls	Treatment
A practice continues past its authorised end date	High	Register with automated expiry alerts	Daily expiry report to the nurse in charge from 1 December 2026
Bed rails applied as routine practice without authorisation	High	Nightly rail audit; authorisation required per person	Rail audit results to committee monthly
Pain mistaken for behavioural disturbance and treated with a psychotropic	High	Mandatory pain assessment before psychotropic	Analgesia trial documented in every case
Secure unit residents lack individual authorisation	High	Individual authorisation on entry	Full secure-unit authorisation review by 31 January 2027
Consent obtained from a person without legal authority	Medium	Authority recorded and verified at authorisation	Quarterly consent authority audit
Elimination plans exist on paper but do not progress	Medium	Quarterly review must record progress	Escalation of practices beyond 12 months

10.1 Dignity of risk

The provider supports each individual to make their own decisions, including decisions that carry risk. Where an individual chooses a course of action a clinician considers unsafe, the discussion, the information given, the individual's decision and any agreed mitigations are documented in the care plan and reviewed at each care plan review. Supporting a person's choice is not a failure of care.

Part 11 Review of this document

This document is reviewed at least every two years, and sooner where a legislative change, a major non-conformity, a serious incident, a coronial finding or a substantive change to practice makes review necessary. The next scheduled review falls due on 1 October 2027.

The review is coordinated by the Clinical Nurse Consultant, Behaviour Support and must involve at least one registered nurse, one worker who applies the document in daily practice, and one consumer representative drawn from the Consumer Advisory Body. The reviewed document is approved by the Clinical Governance Committee and, where the change is material, by the Board.

11.1 Related documents

- MG-CLIN-007 Medication Management Policy
- MG-CLIN-002 Incident Management Policy
- MG-CLIN-015 Dementia Care and Cognitive Support Procedure
- MG-CLIN-019 Pain Assessment and Management Procedure
- MG-CORP-005 Consent and Supported Decision-Making Policy

11.2 Legislative and standards context

- Aged Care Act 2024 (Cth) and the Aged Care Rules made under it
- Strengthened Aged Care Quality Standards, effective 1 November 2025

- Privacy Act 1988 (Cth) and the Australian Privacy Principles
- Work Health and Safety Act 2011 (Cth) and corresponding state legislation
- Quality of Care Principles as they relate to restrictive practices and behaviour support

Part 12 Communication, engagement and supported decision-making

Every person receiving care from Marloo Gardens Aged Care has the right to be told, in a way they understand, what is happening in their care and why. Information provided under this document must be offered in the person's preferred language and format, and must be repeated or restated where comprehension is uncertain. Reliance on a nod or a signature is not evidence of understanding.

12.1 Interpreters and communication support

An accredited interpreter must be engaged where a person's preferred language is not English and the discussion concerns consent, risk, a change to care, an incident affecting them or a complaint. Family members must not be used as interpreters for these discussions except in an emergency, and where that occurs the reason is recorded and the discussion repeated with an accredited interpreter within two business days.

12.2 Supported decision-making

The provider presumes that every person has decision-making capacity. Where capacity is in question, the assessment must be decision-specific, time-specific and documented, and must not be inferred from a diagnosis, age or communication difficulty. A person's supporter assists the person to make and communicate their own decision; a substitute decision-maker acts only where the person cannot make the decision even with support.

12.3 Engagement with supporters and representatives

The provider maintains a current record of each person's nominated supporters and representatives, including the scope of any authority held and the instrument granting it. The record is confirmed with the person at each care plan review and whenever the person's circumstances change.

12.4 Culturally safe practice

Care is delivered in a way that is safe for the person's culture, spirituality, gender, sexuality and life experience. For Aboriginal and Torres Strait Islander people, the provider offers connection to community-controlled health services and to family and community, and records the person's wishes about who is involved in decisions about their care.

Part 13 Emergency, contingency and business continuity

The requirements of this document continue to apply during an emergency. Where an emergency makes ordinary compliance impossible, the provider must record what was done instead, why, and for how long, and must return to ordinary practice as soon as the emergency permits.

13.1 Planned system outages

Where the clinical information system is unavailable, workers record required information on the approved paper forms and enter it into the system within 24 hours of the system being restored. The paper record is retained and scanned into the care record; it is not destroyed after entry.

13.2 Surge, outbreak and staffing shortfall

During an infectious disease outbreak or a staffing shortfall, the Clinical Nurse Consultant, Behaviour Support may authorise temporary variations to non-statutory requirements of this document. Statutory obligations, including notification timeframes, cannot be varied. Every temporary variation is recorded in the variations log with the authorising officer, the reason, the start date and the review date, and is reported to the next committee meeting.

13.3 Evacuation and relocation

Where a person is relocated in an emergency, the receiving service must be given the person's current care plan, medication chart, advance care directive and supporter contact details before or with the person. A relocation is not complete until a handover has been given to and acknowledged by a named clinician at the receiving service.

Part 14 Subcontracted and external service providers

The provider remains fully accountable for care delivered on its behalf by a subcontractor, agency or external practitioner. Engaging another organisation transfers work; it does not transfer accountability.

14.1 Contractual requirements

- Every service agreement must require compliance with this document by name.
- Every agreement must require the subcontractor to report incidents to the provider immediately, and in any event within four hours of becoming aware.
- Every agreement must permit the provider to audit the subcontractor's compliance on reasonable notice.
- Every agreement must require workers supplied to hold current screening and the qualifications the role requires.
- Every agreement must be reviewed before renewal, and no agreement may be renewed where a material non-conformity remains unresolved.

14.2 Monitoring subcontractor performance

The provider reviews the performance of each subcontractor at least annually against the indicators at Part 9, and more frequently where a concern has been identified. The review considers incidents, complaints, workforce screening currency and audit findings. The next scheduled subcontractor review cycle concludes on 31 March 2027.

14.3 Agency and locum workers

An agency worker may not commence a shift until the provider has sighted evidence of current registration where the role requires it, a current worker screening clearance, and completion of the provider's orientation. The orientation attestation is retained for the duration of the engagement and for seven years afterwards.

Part 15 Mapping to the Strengthened Aged Care Quality Standards

This Part records how the requirements of this document give effect to the Strengthened Aged Care Quality Standards. It is used as the starting point for self-assessment and for evidence gathering before a quality audit.

Standard	Outcome	How this document gives effect to it
Standard 1	The individual	Part 1 establishes elimination as the objective and requires consent.
Standard 1	Dignity and choice	5A.2 preserves all other rights for people in secure units.
Standard 3	Care and services	Part 2 requires assessment for unmet need before any restrictive response.
Standard 5	Clinical care	4.3 sets monitoring requirements during physical restraint.
Standard 5	Restrictive practices	Parts 3 to 5 govern authorisation, emergency use and elimination.
Standard 2	The organisation	5.3 requires prevalence reporting to the Board and publicly.

15.1 Evidence for audit

Evidence supporting each mapped outcome is maintained in the evidence library and indexed against the outcome number. Evidence must be current, dated and attributable, and must demonstrate practice rather than intention: a policy document alone is not evidence that the practice occurs.

15.2 Self-assessment

The Clinical Nurse Consultant, Behaviour Support completes a self-assessment against the mapped outcomes at least annually and before any planned quality audit. The self-assessment is endorsed by the Clinical Governance Committee and reported to the Board. The next self-assessment is due by 30 April 2027.

Part 16 Assurance, external review and regulatory engagement

Assurance over this document operates on three lines: the workers and managers who apply it day to day, the Clinical Governance Committee which monitors and challenges performance, and independent review commissioned by the Board. Each line is documented and none substitutes for another.

16.1 Independent review

The Board commissions an independent review of compliance with this document at least every three years, or sooner following a major non-conformity or a serious incident. The reviewer must not have been involved in writing or operating the document. The report goes to the Board unaltered, together with management's response. The next independent review must be completed by 30 September 2027.

16.2 Engagement with the Commission

The provider cooperates fully and promptly with the Aged Care Quality and Safety Commission. Requests for information are acknowledged within one business day and answered within the period specified, or an extension is sought in writing before the period expires. A worker approached directly by the Commission may speak with it freely; the provider must not require a worker to obtain permission first.

16.3 Responding to findings

Where a regulator or reviewer identifies a deficiency, the provider prepares a written response within 14 days setting out what will change, who is accountable, when it will be done and how effectiveness will be measured. Progress is reported to the Board monthly until every action is closed and verified.

16.4 Sanctions and notifiable changes

Where a sanction, a notice to agree, or a notice of non-compliance is issued, the Clinical Nurse Consultant, Behaviour Support must notify the Board Chair on the day it is received, and the provider must tell affected individuals and their supporters what it means for their care within five business days.

Appendix A — Definitions

The following definitions apply throughout this document.

Term	Definition
Restrictive practice	Any practice or intervention that has the effect of restricting the rights or freedom of movement of a person.
Restrictive practices substitute decision-maker	The person or body empowered under State or Territory law to consent to a restrictive practice.
Behaviour support plan	A documented plan describing a person's changed behaviour, its triggers and the strategies that support them.
Least restrictive	The option that achieves the necessary protection while limiting the person's rights as little as possible.
Elimination plan	A documented plan setting out how and by when a restrictive practice will cease.
Emergency use	Use without prior authorisation where there is imminent risk of serious harm and no less restrictive option.

Appendix B — Competency assessment criteria

A worker is assessed as competent when they demonstrate each criterion below by direct observation in the workplace. The assessor records the date, the criterion, the outcome and any remediation required.

Criterion	Evidence sought	Outcome
Identifies each of the five forms of restrictive practice	Correctly classifies five of five scenarios	Competent / Not yet
Assesses for reversible causes before escalation	Observed assessment covers pain, delirium and unmet needs	Competent / Not yet
Applies de-escalation strategies	Observed in simulation or practice	Competent / Not yet
Completes 15-minute monitoring observations	Observed with complete records	Competent / Not yet
Recognises that a lapsed authorisation is a reportable incident	States the 24-hour notification requirement	Competent / Not yet

Appendix C — Internal audit tool

This tool is completed at each internal audit. Each question is answered yes, no or not applicable, with a comment recording the evidence examined. A 'no' answer requires a corrective action with an owner and a due date.

#	Audit question	Evidence examined
1	Does every practice in use have a current authorisation and consent?	Restrictive practices register
2	Does each authorisation record specific alternatives tried and why they were insufficient?	Authorisation records
3	Was every emergency use authorised or ceased within 5 days?	Incident records, register
4	Does every person in the secure unit hold an individual authorisation?	Register, unit records
5	Were 15-minute observations completed during physical restraint?	Observation records
6	Was a pain assessment completed before any psychotropic prescribed for behaviour?	Clinical records
7	Does every authorisation have an elimination plan with a target date?	Authorisation records
8	Were practices in place beyond 12 months escalated to the committee?	Committee minutes

Appendix I — Abbreviations

The following abbreviations are used in this document and in the records it requires. Abbreviations must not be used in clinical entries where the meaning could be ambiguous.

Abbreviation	Meaning
ACQSC	Aged Care Quality and Safety Commission
ATAGI	Australian Technical Advisory Group on Immunisation
CGC	Clinical Governance Committee
CEO	Chief Executive Officer
EN	Enrolled nurse
IDDSI	International Dysphagia Diet Standardisation Initiative
MDT	Multidisciplinary team
PPE	Personal protective equipment
PRN	Pro re nata, meaning as required
RMMR	Residential Medication Management Review
RN	Registered nurse
SDM	Substitute decision-maker
SIRS	Serious Incident Response Scheme
WHS	Work health and safety

Appendix J — Worker acknowledgement

Every worker to whom this document applies must read it and acknowledge that they have done so. Acknowledgement records that the worker has read the document; it does not replace the competency assessment at Appendix B.

Acknowledgements are recorded in the learning management system. A worker who has not acknowledged the current version within 30 days of its publication is reported to their line manager, and a worker who has not acknowledged it within 60 days may not be rostered to a function this document governs until they do.

Acknowledgement statement

I confirm that I have read and understood this document, that I have had the opportunity to ask questions about it, and that I understand my obligations under it. I understand that where I am unsure how this document applies to a situation, I am expected to ask before acting rather than to assume.

Name	Role	Signature	Date

Completed acknowledgement sheets are returned to the document owner and retained for the life of the version plus seven years.

Appendix H — Implementation plan for the current version

The following milestones apply to the implementation of this version. Milestones are tracked by the document owner and reported to the committee monthly until all are closed.

Milestone	Owner	Due
Publish version 4.3 and brief all clinical workers	Document owner	1 Oct 2026
Audit every current authorisation against the new evidence requirement	Clinical Nurse Consultant	30 Nov 2026
Activate the daily expiry report	Digital Systems Manager	1 Dec 2026
Complete the secure unit authorisation review	Clinical Nurse Consultant	31 Jan 2027
Commence nightly bed rail audits	Facility Manager	28 Feb 2027
Report elimination progress to the Board	Clinical Nurse Consultant	30 Jun 2027

Appendix E — Escalation matrix

This matrix sets out who must be told, and how quickly, when a matter arises under this document. Where two rows could apply, the shorter timeframe governs.

Trigger	Escalate to	Within
Use of a restrictive practice without authorisation	Facility Manager and incident process	Immediately
Any use of seclusion	Chief Executive Officer and Board Chair	Immediately
Authorisation reaching its end date	Clinical Nurse Consultant	3 days before expiry
Deterioration during physical restraint	Registered nurse in charge	Immediately
Substitute decision-maker withdraws consent	Clinical Nurse Consultant	Same day
Practice in place beyond 12 months	Clinical Governance Committee	At the next meeting

Appendix F — Forms and templates

The following forms support this document. Forms are version-controlled and must be obtained from the intranet rather than copied from a local drive.

Form	Purpose	Retained by
MG-F-011a Restrictive practice authorisation	Records the practice, purpose, alternatives, duration and consent	Clinical Nurse Consultant
MG-F-011b Behaviour support plan	Records triggers, strategies and review dates	Clinical Nurse Consultant
MG-F-011c Monitoring observation chart	Records 15-minute observations during restraint	Registered nurse
MG-F-011d Elimination plan	Records the target cessation date and the steps toward it	Clinical Nurse Consultant
MG-F-011e Emergency use record	Records the circumstances and the follow-up obligations	Facility Manager

Appendix G — Frequently asked questions

These questions arose during consultation on the current version and are reproduced with the answers given.

Question	Answer
Are bed rails a restrictive practice?	Yes, whenever they prevent a person leaving the bed. They require assessment, consent, authorisation and monitoring like any other physical restraint.
A resident's daughter asks us to lock her mother's door at night. Can we?	No. That would be seclusion, which is prohibited here in all circumstances. Explain the alternatives available and record the conversation.
The resident is agitated and there is no authorisation. What can I do right now?	Use de-escalation and address unmet needs first. Emergency use is permitted only where there is imminent risk of serious harm, and triggers the Part 4 obligations immediately.
Does living in the secure unit mean environmental restraint is already consented to?	No. Each person needs their own authorisation and consent, reviewed at least every three months.
The authorisation expired yesterday but nothing else has changed. Is that a problem?	Yes. Use beyond the authorised end date is unauthorised use and a Priority 1 reportable incident, even if the clinical situation is unchanged.

Appendix D — Improvement actions arising from the current review

The following actions were agreed at the most recent review of this document and are tracked in the continuous improvement register until closed.

Ref	Action	Responsible	Due
R1	Implement a daily authorisation-expiry report to the nurse in charge	Digital Systems Manager	1 Dec 2026
R2	Review every secure unit resident's individual authorisation	Clinical Nurse Consultant	31 Jan 2027
R3	Escalate and re-examine all practices in place beyond 12 months	Clinical Nurse Consultant	31 Jan 2027
R4	Introduce a nightly bed rail audit	Facility Manager	28 Feb 2027
R5	Deliver pain assessment refresher training to all nurses	Clinical Educator	31 Mar 2027
R6	Publish the annual restrictive practices reduction report	Clinical Nurse Consultant	31 Aug 2027